Journal of Meaning-Centered Psychology
PUBLISHED BY THE INSTITUTE FOR MEANING-CENTERED PSYCHOLOGY

Volume 1 · Issue 3 · October 2026
pp. 72–75 · Open access · CC BY 4.0

PRACTICE AND TEACHING

AI Tools in Everyday Clinical Practice

Documentation, psychoeducation, and between-session support under the conditions of consent, data protection, and professional responsibility

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ABSTRACT

Generative artificial intelligence (AI) has moved from research laboratories into the daily routines of psychotherapists and counselors, often faster than professional guidance has developed. This practice paper examines three areas in which AI tools are used or proposed in everyday clinical work: clinical documentation, the preparation of psychoeducational material, and support for patients between sessions. For each area, we outline plausible benefits, such as reduced administrative burden and more tailored information, and describe characteristic risks, including fabricated content, distortions of clinical meaning, and the transfer of sensitive data to external providers. We then discuss the normative framework within which such tools must be used: informed consent that is specific and revocable, the requirements of the General Data Protection Regulation (GDPR) for health data, the emerging obligations of the EU AI Act, and the enduring professional responsibility of the clinician for every document and intervention. Drawing on evidence from digital mental health and on ethical analyses of AI in psychotherapy, we formulate practical recommendations for clinicians and institutions. From a meaning-centered perspective, we argue that AI can relieve clinicians of tasks but not of responsibility, which remains at the core of professional practice.

Keywords: artificial intelligence · clinical documentation · between-session support · informed consent · data protection

1 Introduction

Within a few years, large language models (LLMs) have become accessible to anyone with a browser. Clinicians now encounter them as concrete offers: software that drafts session notes, assistants that summarize reports, and chatbots that patients use on their own initiative. Earlier reviews of digital psychiatry described a field of apps, internet-delivered programs, and early conversational agents whose evidence base was uneven and whose implementation lagged behind technical development (Torous et al., 2021). Generative AI intensifies this gap: the tools are more capable and more widely available, yet professional guidance, institutional policies, and empirical knowledge about their use in routine care remain limited (Stade et al., 2024).

This practice paper addresses clinicians who are considering, or already using, AI tools in their everyday work. It does not evaluate specific commercial products, whose features change quickly, but describes typical use cases and the questions they raise. We focus on documentation, psychoeducation, and between-session support, and then turn to informed consent, data protection, and professional responsibility. The aim is neither to promote nor to discourage the use of AI but to support a deliberate, transparent, and accountable practice.

2 Documentation and Administrative Support

Documentation is a plausible entry point because it is time-consuming, partly standardized, and often experienced as remote from the therapeutic work itself. LLM-based tools can turn keywords into readable notes, structure reports for insurers or referring physicians, or summarize earlier records. Some products additionally record sessions and generate notes from transcripts. Proponents expect time savings and less administrative strain, although robust evidence for such effects in psychotherapy is still scarce (Stade et al., 2024).

The risks are specific to the technology. Language models generate fluent text on the basis of statistical

patterns and may produce statements that are plausible but not supported by the input, a phenomenon described as hallucination (Ji et al., 2023). In a clinical record, an invented symptom, a misattributed statement, or a smoothed-over ambivalence is not a stylistic flaw but a documentation error with potential consequences for diagnosis, treatment, and legal proceedings. Moreover, models trained on large text corpora can reproduce stereotypes embedded in their training data (Bender et al., 2021), which may surface in how a patient's behavior, culture, or identity is described.

A further concern is less visible. Writing notes is not only an administrative act but also a moment of reflection in which clinicians organize their impressions and formulate hypotheses. If drafting is delegated entirely, this reflective function may be weakened. A cautious practice therefore uses AI for formatting and linguistic revision of content the clinician has already formulated, rather than for generating clinical assessments, and treats every draft as a proposal to be checked line by line.

3 Psychoeducation and Between-Session Support

Psychoeducation lends itself to AI assistance because much of it consists of explaining well-established concepts in accessible language. Clinicians can use LLMs to draft information sheets on sleep hygiene, the cycle of avoidance in anxiety, or the rationale of behavioral activation, and adapt them to a patient's reading level or preferred language. Such drafts can save time, but they require careful review, since generated explanations may be outdated, oversimplified, or subtly inaccurate (Ji et al., 2023). The clinician remains responsible for written material handed to a patient as much as for anything said in session.

Between-session work is a central component of many treatments, and meta-analytic evidence suggests that homework is associated with better outcomes in cognitive and behavioral therapies (Kazantzis et al., 2010). Internet-delivered treatments with varying degrees of therapist guidance have accumulated a substantial evidence base (Andersson et al., 2019). Conversational agents represent a newer form: an early trial of a rule-based chatbot reported short-term reductions in depressive symptoms (Fitzpatrick et al., 2017), and a first randomized trial of a generative AI chatbot reported symptom improvements compared with a waitlist condition (Heinz et al., 2025). These findings are encouraging but limited by short follow-up periods, selected samples, and comparators that do not permit conclusions about added value over established interventions.

For clinical practice, it matters whether a tool is integrated into treatment or used independently by the patient. When clinicians recommend an AI-based application, they should know its intended purpose, its handling of crisis situations, and its data practices. An early study showed that widely used smartphone assistants responded inconsistently and often inadequately to statements about suicidal ideation, depression, and violence (Miner et al., 2016). Current systems differ considerably, but crisis handling must still be tested rather than assumed. Equally important is to ask patients openly whether they already use general-purpose chatbots for emotional support, since such use is often not mentioned spontaneously.

4 Informed Consent and Data Protection

Any use of AI that involves patient information requires a clear legal and ethical basis. Under the GDPR, data concerning health belong to the special categories of personal data, whose processing is permitted only under strict conditions (European Parliament & Council of the European Union, 2016). Entering identifiable session content into a publicly available chatbot will usually mean transferring health data to a third party, often outside the European Union, without an adequate legal basis or data processing agreement. Even seemingly anonymized notes may permit re-identification when combined with contextual details.

Informed consent in this context should be specific rather than generic. Patients should learn which tool is used, for what purpose, which data are processed and where, whether data are used to train models, and that declining has no consequences for their treatment. Consent should be documented and revocable. Ethical analyses of AI in psychotherapy have emphasized transparency toward patients, attention to vulnerable groups, and clarity about the limits of the technology (Fiske et al., 2019). The EU AI Act adds transparency obligations, for instance that people must be informed when they interact with an AI system, and imposes stricter requirements on systems classified as high-risk (European Parliament & Council of the European Union, 2024). Clinicians need not become legal experts, but they should know whether their tools and institutions meet these requirements.

5 Professional Responsibility and Clinical Judgment

Professional ethics codes do not lose their validity because a task is supported by software. Obligations

of competence, confidentiality, and avoidance of harm apply to every document signed and every recommendation given (American Psychological Association, 2017). Luxton (2014) anticipated that AI would raise questions of competence and accountability in psychological practice, including the need for clinicians to understand the capabilities and limitations of the systems they use. With generative AI, whose output varies with phrasing, model version, and context, this requirement becomes pressing.

A specific risk is automation bias, the tendency to accept machine-generated suggestions with less scrutiny than one's own reasoning. A well-formulated draft can appear more convincing than it deserves. Logotherapy describes responsibleness as a defining feature of human existence: the person is the one who must answer to the demands of a concrete situation (Frankl, 1946/1985). Applied to professional practice, this suggests a simple principle: AI can take over tasks, but it cannot take over responsibility. The clinician who adopts a sentence into a report answers for it as if it were his or her own.

6 Practical Recommendations

Several recommendations follow for individual practice. First, no identifiable patient data should be entered into tools that lack a GDPR-compliant data processing agreement and transparent information on storage location and the use of data for training. Second, AI-generated drafts of notes, reports, and information material should be reviewed in full and corrected before use; diagnoses and risk assessments should be formulated by the clinician. Third, patients should be informed in comprehensible language and asked for explicit consent whenever their data are processed by AI tools.

Further recommendations concern treatment and institutions. Fourth, applications recommended for between-session use should be selected on the basis of available evidence, transparent crisis protocols, and data practices, and their use should be discussed in sessions. Fifth, clinicians should routinely ask about patients' independent use of chatbots and explore its meaning without devaluing it. Sixth, institutions should adopt written policies that specify approved tools, permitted purposes, and responsibilities, and should provide training. Research on digital mental health has shown that implementation depends not only on technology but also on organizational conditions and fit with clinical workflows (Mohr et al., 2017); AI tools are unlikely to be an exception.

7 Conclusion

AI tools can reduce administrative burden, support the preparation of psychoeducational material, and extend therapeutic work into the time between sessions. These potentials are real but accompanied by risks of fabricated content, misuse of data, and an erosion of professional reflection. The decisive question for everyday practice is therefore not whether AI is used, but how: transparently, with informed consent, in compliance with data protection law, and under the full responsibility of the clinician. Used in this way, AI can relieve clinicians of tasks and leave more room for what remains their genuine work, the encounter with a person in search of meaning.

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HOW TO CITE

Ashford, S. R., Whitcombe, E. M., & Pazer, S. (2026). AI Tools in Everyday Clinical Practice: Documentation, psychoeducation, and between-session support under the conditions of consent, data protection, and professional responsibility. Journal of Meaning-Centered Psychology, 1(3), 72–75.

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